

Treatment for abnormal cervical cells

If your cervical screening results show abnormal cells, you may have treatment to remove or destroy them.

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Treating abnormal cells after cervical screening

If [cervical screening](#) finds abnormal cells on [the cervix](#), you may be offered treatment.

Abnormal cells such as [CIN](#) usually affect a small area where the outer cervix meets the cervical canal. Most treatments aim to remove or destroy only this small area of abnormal cells. This means nearby healthy areas of cervix are not likely to be damaged.

Image: Area of cervix treated

Types of treatment

There are different types of treatment. You usually only need one treatment to remove the abnormal cells completely. Your doctor will explain what type of treatment they suggest for you.

This may depend on the treatments your local hospital can provide. It may also depend on the type of abnormal cells and the area of the cervix that is affected.

Treatments that remove the abnormal cells

Treatments that remove the abnormal area include:

A [large loop excision of the transformation zone \(LLETZ\)](#) removes abnormal cells from the cervix using a thin loop-shaped tool.

Cone biopsy

A [cone biopsy](#) is a small operation to remove an area of the cervix.

After these treatments, the removed area of tissue is sent to a laboratory. It is checked to confirm the type of abnormal cell changes.

Sometimes, a type of surgery called a [hysterectomy](#) is used to remove the whole cervix and womb. This is not a common treatment for abnormal cells. Your doctor will explain if they think it is right for you.

Treatments that destroy the abnormal cells

Treatments that destroy the cells in the abnormal area include:

- laser therapy
- cold coagulation
- cryotherapy.

Laser therapy

This treatment uses a laser beam to burn away the abnormal cells. It is also called laser ablation. It is usually done under a general anaesthetic and you may need to stay overnight in hospital.

Cold coagulation

Despite the name, this treatment uses heat to destroy the abnormal cells. Some local anaesthetic is used to numb the cervix. Then, a small heated probe is placed onto any abnormal areas to burn them away.

Cryotherapy

A small probe is used to freeze the abnormal cells on the cervix. You may be given a local anaesthetic, but often this is not needed.

What to expect during treatment

Getting ready for any of these treatments is similar to having a [colposcopy](#). When you are ready, you undress from the waist down. You then lie on your back on an examination couch. Some clinics have couches with foot or leg supports that you can rest your legs up on. You will be asked to lie with your knees bent and apart.

The doctor or nurse gently puts an instrument called a speculum into the vagina. This holds the vagina open so that they can see the cervix. It should not hurt but sometimes it can feel uncomfortable.

You may find this type of treatment upsetting or embarrassing. Your healthcare team will try to help. Let them know how you are feeling and tell them if you have any questions or worries. If you want to bring someone with you for support during the treatment, this can usually be arranged.

After your treatment

If you have a general anaesthetic, you will stay in hospital for the day or overnight after your treatment. If you have a local anaesthetic, you will be able to go home the same day.

You may feel fine after your treatment. But you may feel slightly unwell for a few hours after the local anaesthetic. It is a good idea to have the day off work, in case you need to go home and rest. You may want to arrange someone to help you home.

You may have some period-like pains for a few hours after the treatment. Some bleeding or discharge after treatment is normal. This usually stops within 4 weeks, but may last up to 6 weeks. The bleeding should not be heavier than a moderate period and should get steadily lighter.

You should contact your GP or the clinic where you had your treatment if:

- the bleeding gets heavier – for example, completely soaking a pad within 2 hours
- the discharge smells unpleasant
- you have a fever or temperature
- you have severe pain
- you are worried for another reason.

It will take a few weeks for the cervix to heal. Your doctor or nurse will probably advise you not to have penetrative sex for at least 4 weeks after your treatment. This allows the cervix to heal properly. You may also be advised not to use tampons or swim for 4 weeks, and to wash or shower rather than have a bath.

Your treatment should not affect your ability to enjoy sex after the cervix has healed.

Fertility and pregnancy after treatment

Your treatment should not affect your ability to get pregnant.

Very rarely, the cervix can become tightly closed after treatment. This is called stenosis. It may make it harder for sperm to enter the womb and so can affect your chances of getting pregnant naturally. Tell your doctor if your periods stop after treatment or you have worse period pain than usual.

Removing some of the cervix may also make it slightly weaker. This depends on how much needs to be removed. You may be more likely to give birth early if the cervix is weakened. Some women may be referred to a local specialist maternity service for closer monitoring during pregnancy. Your doctor can tell you more about this.

About our information

References



Below is a sample of the sources used in our cervical cancer, cervical screening and CIN information. If you would like more information about the sources we use, please contact us at canцерinformationteam@macmillan.org.uk

GOV.UK. Cervical screening: programme overview. Updated 18 November 2019. Available from www.gov.uk/guidance/cervical-screening-programme-overview (accessed March 2020).

GOV.UK. Colposcopic diagnosis, treatment and follow up. Updated 5 February 2020. Available from www.gov.uk/government/publications/cervical-screening-programme-and-colposcopy-management/3-colposcopic-diagnosis-treatment-and-follow-up (accessed April 2020).

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Reviewers



This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer. It has been approved by Senior Medical Editor, [Professor Nick Reed](#), Consultant Clinical Oncologist.

Our cancer information has been awarded the [PIF TICK](#). Created by the Patient Information Forum, this quality mark shows we meet PIF's 10 criteria for trustworthy health information.

The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as 'you' so that everyone feels included. Where clinically necessary we use the terms 'men' and 'women' or 'male' and 'female'. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information [here](#).

Date reviewed

Reviewed: 01 April 2021 | Next review: 01 April 2024

ⓘ This content is currently being reviewed. New information will be coming soon.

Our cancer information meets the [PIF TICK](#) quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about [how we produce our information](#).

How we can help

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.



Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.



Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don't want to call.



Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.



Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help